

YKP3089C017 Protocol Operations Packet

Study synopsis

YKP3089C017 is a Phase 2b dose-response trial of YKP3089 added to 1 to 3 existing antiepileptic drugs (AEDs) in adults with uncontrolled partial-onset seizures.

Item	What the protocol says
Full title	A Multicenter, Double-Blind, Randomized, Placebo-Controlled, Dose-Response Trial of YKP3089 as Adjunctive Therapy in Subjects with Partial Onset Seizures, with Optional Open-Label Extension
Sponsor and identifiers	SK Life Science Inc. IND 76,809. EudraCT 2013-001858-10
Version in this packet	Amendment 3, June 17, 2019 (original June 12, 2013; Amendment 1 January 7, 2014; Amendment 2 March 20, 2015)
Phase and indication	Phase 2b. Partial-onset epilepsy
Size	About 100 sites in multiple countries. About 520 screened to randomize 400, 100 per arm
Primary objective	Determine the effective dose range of YKP3089 as adjunctive therapy for partial seizures
Other objective	Evaluate safety and tolerability in the partial epilepsy population
Design	8-week prospective baseline, then 18 weeks double-blind (6-week titration plus 12-week maintenance), then either a 3-week blinded taper with follow-up or a 2-week blinded conversion into the open-label extension
Arms	Placebo, 100 mg, 200 mg or 400 mg once daily in the morning. 1:1:1:1 through the interactive web response system (IWRS), permuted blocks
Study drug	50 mg and 100 mg tablets and matching placebo, identical in appearance. Blister cards of 7 days plus 2 spare days while blinded. Bottles of 100 tablets in open label

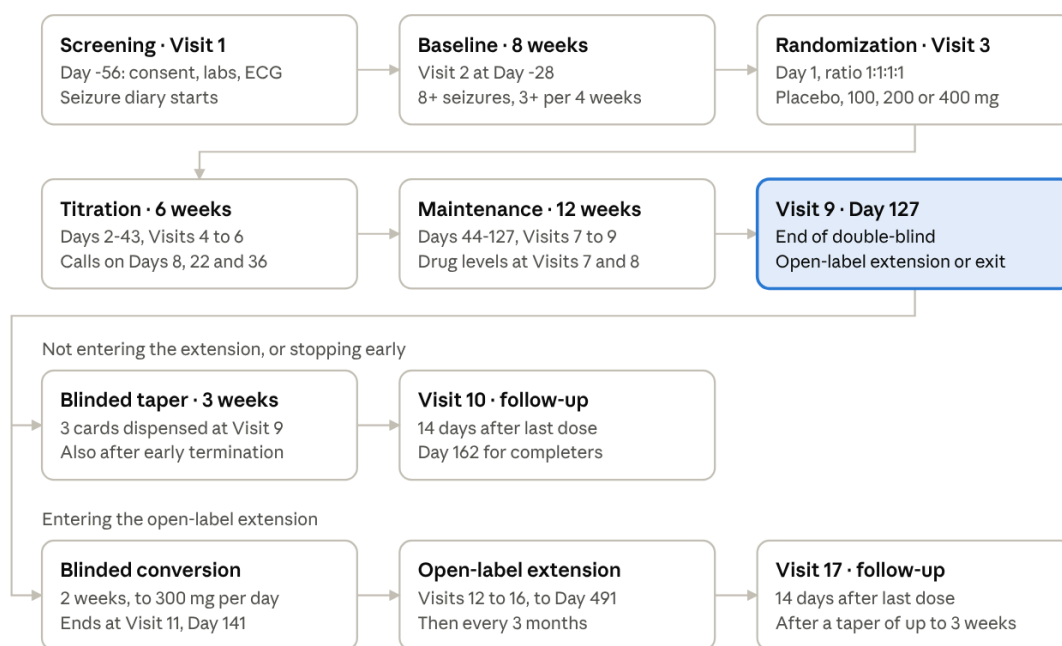
Item	What the protocol says
Who qualifies, in one line	Adults 18 to 70, at least 40 kg, on stable doses of 1 to 3 AEDs, with at least 8 qualifying partial seizures in the 8-week baseline
Primary endpoint, United States and rest of world	Percent change from baseline in seizure frequency per 28 days over the double-blind period. Counts simple partial motor, complex partial and secondarily generalized seizures
Primary endpoint, Europe, Australia, New Zealand, South Africa	Responder rate: 50% or greater reduction from baseline during the maintenance phase
Other efficacy	75%, 90% and 100% responder rates. Change by seizure subtype. Clinical Global Impression of Change (CGIC). Quality of Life in Epilepsy (QOLIE-31-P)
Drug levels	Trough concomitant AED levels at Visits 3, 7 and 8 for oxcarbazepine, topiramate, carbamazepine, valproate, lamotrigine, lacosamide and levetiracetam. YKP3089 plasma levels at Visits 7 and 8
Safety assessments	Adverse events, orthostatic vital signs, physical and neurologic exams, hematology, chemistry, urinalysis, 12-lead ECG, Columbia-Suicide Severity Rating Scale (C-SSRS)
Oversight	An independent Data Monitoring Committee reviews unblinded safety data at 4 milestones. An independent epilepsy group confirms seizure classification before any subject is randomized
Serious adverse events	Fatal or life-threatening: phone the sponsor's designee immediately. All SAEs: SAE Report Form by fax or email within 24 hours of awareness. Written narrative within the following 48 hours. Report to the IRB
Adverse event window	From the first study procedure through 30 days after the last dose
Seizures as adverse events	A seizure alone is not an AE. Record one only for a measurable increase over the subject's usual frequency or duration, or a pattern distinguishable from usual

Item	What the protocol says
Rash	Any rash, seen or reported by phone, needs a prompt unscheduled visit with hematology and chemistry to evaluate for drug hypersensitivity
Emergency unblinding	The investigator can unblind in IWRS without prior contact when treatment knowledge is essential for care, then documents it and informs the Medical Monitor promptly
Withdrawal reasons	Consent withdrawn, non-adherence, intolerable AE, pregnancy, lost to follow-up. Discharge procedures are still performed
Time on study without the extension	About 31 weeks: 8 baseline, 18 treatment, 3 taper, 2 follow-up

Why two primary endpoints matter to you: both are computed from the seizure diary. Every missing diary day weakens both analyses, so diary review is the most important thing you do at each visit.

Study schema

Visit 9 is the fork: taper and exit, or convert to open label



study schema · 6 common steps, 2 exits

Every subject follows the top two rows. At Visit 9 a subject either tapers and returns for Visit 10, or converts under blind to 300 mg per day and continues in the extension until Visit 17.

Visit schedule at a glance

Day 1 is the randomization visit and the first dose is taken on Day 2. Every visit includes vital signs, concomitant medications and the seizure diary, so the last column lists only what is added.

Visit	Target day	Window	Phase	What sets it apart
1	-56	±2 days	Screening	Consent, history, full physical and full neurologic exams, height, triplicate ECG, serum pregnancy test, labs, urinalysis, Diagnostic Review Form, first diary
2	-28	±2 days	Baseline	Diary review and adverse events only. No C-SSRS at this visit
3	1	±2 days	Randomization	Baseline seizure count, eligibility re-check, QOLIE-31-P, ECG, urine pregnancy test, labs, urinalysis, concomitant AED levels, randomize, first cards
Call	8	Not stated	Titration	Tolerability, adverse events, compliance
4	15	±2 days	Titration	Brief neurologic exam, C-SSRS, first drug accountability
Call	22	Not stated	Titration	Tolerability, adverse events, compliance
5	29	±2 days	Titration	Weight

Visit	Target day	Window	Phase	What sets it apart
Call	36	Not stated	Titration	Tolerability, adverse events, compliance
6	43	±2 days	End of titration	ECG, labs
7	71	±3 days	Maintenance	Weight, two timed YKP3089 samples, concomitant AED levels
8	99	±3 days	Maintenance	Two timed YKP3089 samples, concomitant AED levels
9	127	±3 days	End of double-blind, or early termination	Full neurologic exam, QOLIE-31-P, CGIC, ECG, urine pregnancy test, labs, urinalysis, weight. Taper cards or conversion supplies
10	162, or 14 days after last dose	±3 days	Follow-up, subjects not entering the extension	Full physical exam, serum pregnancy test, labs, urinalysis, weight. No drug dispensed
11	141	±2 days	Extension, conversion ends	Brief neurologic exam. Open-label bottles start
12	155	±2 days	Extension	ECG, urine pregnancy test, labs
13	239	±7 days	Extension	Weight, brief neurologic exam

Visit	Target day	Window	Phase	What sets it apart
14	323	±7 days	Extension	Urine pregnancy test, labs, urinalysis
15	407	±7 days	Extension	Weight, brief neurologic exam
16	491	±7 days	Extension year one, or termination	Full physical and full neurologic exams, ECG, urine pregnancy test, labs, urinalysis, weight, benefit decision
18 onward	Every 3 months after Visit 16	±7 days	Extension beyond year one	Every 6 months: weight, brief neurologic exam, urine pregnancy test, labs. Every 12 months: full physical exam, ECG, urinalysis
17	14 days after last dose	±3 days	End-of-study follow-up, extension	Full neurologic exam, serum pregnancy test, labs, urinalysis, weight. No drug dispensed

Adverse events are reviewed from Visit 2 in the protocol table, C-SSRS at every visit except Visit 2, and drug accountability from Visit 4. Schedule target dates from Day 1 rather than from the last actual visit, so one late visit does not push every later visit late. That is standard practice, not protocol text, so confirm it with the monitor.

Dosing rules

You and the subject never learn the double-blind dose, even during conversion: IWRS assigns every card, and you never pick a card strength yourself.

Double-blind titration (mg per day)

Target dose	Week 1	Week 2	Week 3	Week 4	Week 5	Week 6
100	50	100	100	100	100	100
200	50	100	150	200	200	200

Target dose	Week 1	Week 2	Week 3	Week 4	Week 5	Week 6
400	50	100	150	200	300	400

Dose changes while blinded

- Week 1: no reduction allowed. A subject with significant intolerance stops study drug.
- Weeks 2 to 6, first tolerability problem: one reduction. 50 mg if taking 100, 150 or 200 mg. 100 mg if taking 300 or 400 mg. The subject stays there 7 to 13 days.
- After that the investigator may resume 50 mg weekly increases toward target, but not after week 6.
- If the re-titration is not tolerated: one more 50 mg reduction, allowed through the end of week 8. Never two reductions in a row. No changes after week 8.
- During titration the investigator may hold the daily dose up to 2 days per week.
- Every change happens at a scheduled or unscheduled visit. The subject returns all cards and IWRS issues replacements.
- Evening dosing is allowed for tolerability. Splitting into 2 doses is allowed only in open label.

Taper, same schedule blinded and open label (mg per day)

Final dose	Week 1	Week 2	Week 3
Placebo, 50 or 100	0	0	0
150 or 200	100	0	0
250 or 300	200	100	0
350 or 400	300	200	100

In the blinded taper every subject receives 3 cards at Visit 9, whatever their dose, so the blind holds. In an open label the investigator may stop the drug at once for safety.

Blinded conversion to open label, Visit 9 to Visit 11 (mg per day)

Double-blind dose group	Week 1 from card	Week 1 from bottle	Week 2 from card	Week 2 from bottle	From Visit 11
Placebo or 50	0	2 x 50	0	4 x 50	300 from bottle
100	100	2 x 50	100	4 x 50	300 from bottle
150 or 200	100	2 x 50	100	4 x 50	300 from bottle
250 or 300	200	2 x 50	100	4 x 50	300 from bottle
350 or 400	200	2 x 50	100	4 x 50	300 from bottle

What the subject actually does: 4 tablets from the card every morning, plus 2 bottle tablets in week 1 and 4 in week 2. From Visit 11, three 100 mg tablets from the bottle. All card tablets must be taken. Adjust only with bottle tablets: up or down 50 to 100 mg in week 1; down 50 to 200 mg or up at most 100 mg in week 2.

Open-label extension

- Target 300 mg per day. Range 50 to 400 mg. Go above 300 only after 300 has been reached.
- Change by 50 or 100 mg per week, faster or slower if clinically indicated.
- A subject who cannot tolerate 50 mg withdraws. YKP3089 is never given as monotherapy.

Other medications

- Concomitant AEDs: total daily dose and frequency stay fixed through the double-blind period. Only the timing or size of an individual dose may shift. Changes are allowed during the taper and in open labels.
- Rescue benzodiazepines, not diazepam: once during baseline and twice during the treatment phase. Two doses within 24 hours count as one rescue.
- Not allowed during the study or within 30 days before Visit 1: clopidogrel, fluvoxamine, amitriptyline, clomipramine, bupropion, methadone, ifosfamide, cyclophosphamide, efavirenz, natural progesterone, diazepam, phenytoin, phenobarbital, other investigational drugs or devices.
- Vigabatrin: not allowed during the study or within 1 year before Visit 1.
- Contraception for women of childbearing potential runs through 30 days after the last dose. Hormonal contraception alone is not adequate. Male subjects have no contraception requirement.